



Division of Provider Services & Quality Assurance
Office of Long-Term Care
P.O. Box 8059, Slot S404, Little Rock, AR 72203-1437

Notice of Serious Incident

Case Number: 032061

Date of Incident: 08/12/2025

Date Received: 08/13/2025

Facility Name: Millcreek of Arkansas PRTF

Incident Type: DPSQA - Incident & Accident - OLTC

Case Status: Under Review

DPSQA - Incident & Accident - OLTC

Report Description: [REDACTED] reported, during a verbal altercation at the school, [REDACTED] allegedly tried to choke him and attempted to bite him. This incident was discussed with [REDACTED] and she denies the allegations. At this time [REDACTED] has been placed on administrative leave pending the results of the investigation.

Arkansas State Agency

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 3002	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING: _____	(X3) DATE SURVEY COMPLETED C 08/15/2025
NAME OF PROVIDER OR SUPPLIER HABILITATION CENTER, LLC		STREET ADDRESS, CITY, STATE, ZIP CODE 1810 INDUSTRIAL DRIVE FORDYCE, AR 71742		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
- 000	Initial Comment The facility was not in compliance with Title 20, Chapter 10. Office of Long Term Care, Division of Provider Services and Quality Assurance, Department of Human Services Part 415. Psychiatric Residential Treatment Facility Providers. Case 032030 Case 032061	- 000		
-501 SS=D	Use of restraints (a)(1) The United States Children's Health Act of 2000 (Public Law No. 106-310) imposes procedural reporting and training requirements regarding the use of restraints and involuntary seclusion in Psychiatric Residential Treatment Facilities that provide inpatient psychiatric services for children who are under twenty-one (21) years of age. (2) Satellite training by the Centers for Medicare & Medicaid Services may be accessed on the Centers for Medicare & Medicaid Services website at www.cms.gov. Federal regulations are located at 42 C.F.R. § 483, Subpart G (2001). (3) The use of mechanical restraints and seclusions are strictly prohibited. (b) A Psychiatric Residential Treatment Facility will report monthly on an approved Office of Long-Term Care template, to the State-designated Protection and Advocacy system, the Office of Long-Term Care, and the Resident's PASSE, or the Resident's sending state the following data: (1) Physical restraints used within the Psychiatric Residential Treatment Facility, in the aggregate and by client; (2) Chemical restraints used within the Psychiatric Residential Treatment Facility, in the	-501		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE



CEO

9/5/25

STATE FORM

6899

XKTK11

If continuation sheet 1 of 17

Arkansas State Agency

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-501	Continued From page 1 aggregate and by client; (3) Sedating PRNs within the Psychiatric Residential Treatment Facility, in the aggregate and by client; and (4) Ratio of chemical restraints to sedating PRNs, in the aggregate and by client. (c) An order for restraint must not be written as a standing order or on an as-needed basis. (d) Restraint must not result in harm or injury to the Resident and must be used only: (1) To ensure the safety of the Resident or others during an emergency safety situation; and (2) Until the emergency safety situation has ceased, and the Resident's safety and the safety of others can be ensured, even if the restraint order has not expired. (e) Restraint must not be used simultaneously. (f) Orders for restraint must be ordered by a physician, or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint or seclusion and trained in the use of emergency safety interventions. (g) If the Resident's treatment team physician is available, only he or she can order a restraint. (h) A physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order a restraint must order the least restrictive emergency safety intervention that is most likely to be effective in resolving the emergency safety situation based on consultation with staff. (i)(1) If the order for restraint is verbal, the verbal order must be received by a registered nurse or other licensed staff such as a licensed practical nurse, while the emergency safety intervention is being initiated by staff or	-501		

Arkansas State Agency

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-501	Continued From page 2 immediately after the emergency safety situation ends. (2) The physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint must verify the verbal order in a signed written form in the Resident's record. (3) The physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint must be available to staff for consultation, at least by telephone, throughout the period of the emergency safety intervention. (j) Each order for restraint must: (1) Be limited to no longer than the duration of the emergency safety situation; and (2) Under no circumstances exceed four (4) hours for Residents who are between eighteen (18) years of age and twenty-one (21) years of age; two (2) hours for Residents who are between nine (9) years of age and seventeen (17) years of age; or one (1) hour for Residents who are under nine (9) years of age. (k) Within one (1) hour of the initiation of the emergency safety intervention a physician, or other licensed practitioner trained in the use of emergency safety interventions and permitted by the state and the Psychiatric Residential Treatment Facility to assess the physical and psychological well-being of Residents, must conduct a face-to-face assessment of the physical and psychological well-being of the Resident, including but not limited to: (1) The Resident's physical and psychological status; (2) The Resident's behavior; (3) The appropriateness of the intervention measures; and (4) Any complications resulting from the	-501		

Arkansas State Agency

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-501	Continued From page 3 intervention. (l) Emergency safety intervention: An emergency safety intervention must be performed in a manner that is safe and proportionate and that is appropriate to the severity of the behavior and to the Resident's chronological and developmental age; size; gender; physical, medical, and psychiatric condition; and personal history (including any history of physical or sexual abuse). (m) Staff must document the intervention in the Resident's record. That documentation must be completed by the end of the shift in which the intervention occurs. If the intervention does not end during the shift in which it began, documentation must be completed during the shift in which it ends. Documentation must include all of the following: (1) Each order for restraint; (2) The time the emergency safety intervention actually began and ended; (3) The time and results of the one (1) hour physical and psychological assessment required via face-to-face of the well-being of the Resident as required above; (4) The emergency safety situation that required the Resident to be restrained; and (5) The name of staff involved in the emergency safety intervention. (n) The Psychiatric Residential Treatment Facility must maintain a record of each emergency safety situation, the interventions used, and their outcomes. (o) The physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint must sign the restraint or seclusion order in the Resident's record as soon as possible.	-501		

Arkansas State Agency

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-501	Continued From page 4 Authority. Arkansas Code §§ 20-76-201 et seq., and 25-10-129. Authority. Arkansas Code §§ 20-76-201 et seq., and 25-10-129. This ELEMENT is not met as evidenced by: Based on observation of video, interview and record review, the facility failed to obtain a physician order for a restraint and failed to document the required documentation for physical restraints for 2 (Client 1 and Client 2) of 3 residents reviewed. Client #1 On 8/14/25 at 2:42 PM a review of video surveillance was conducted for a restraint which occurred on 08/01/2025 in the unit's dayroom. The video footage revealed between 2:58:20 PM and 3:01:23 PM Client #1 was repeatedly moving toward another client in an aggressive manner. Client #1 grabbed a broom and shook it at the other client. At 3:01:28 PM Client #1 was standing still and did not have the broom in the Client's hands. Behavioral Health Associate (BHA) #1, walked into the view of the camera. At 3:02:37 PM BHA #1 was observed grabbing Client #1 with both hands on the client's shoulders, turned Client #1 around and pulled both of Client #1's arms behind the Clients' back. BHA #1 walked from the dayroom to the client's room, holding Client #1's arms behind the clients back. Between 3:02:37 PM and 3:309 PM BHA #1 lifted Client #1 off the ground. A review of Client #1's file on 08/14/2025 did not reveal a physician order for a restraint and failed to reveal the required restraint documentation for the physical restraint conducted by BHA #1 on	-501	501 Step #1: Corrective Action - On 8/8/2025, the employee involved in the incident with Client #1 was placed on administrative leave and was terminated on 9/4/2025. - On 8/14/2025, the employee involved in the incident with Client #2 was placed on administrative leave. - On 9/17/2025, the deficiencies noted will be a topic of discussion in the all employee town halls. - By 9/19/2025, all BHAs, Nurses, and teachers will be in-serviced on a patient escort being considered a restraint and ensuring all required documentation for that restraint is completed. Step #2: Identification of others with a potential to be affected by deficient practice: - On 8/8/2025, through census review, there were 164 other clients within the PRTF with the potential to be affected by the deficient practice noted. Of the 164, 16 clients were in the unit of the BIIA involved in the incident with Client #1. - On 8/12/2025, through census review, there were 170 other clients within the PRTF with the potential to be affected by the deficient practice noted. Of the 170, 14 clients were in the classroom where the incident with Client #2 took place.	

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-501	Continued From page 5 08/01/2025. On 08/14/2025 at 2:42 PM, an interview was conducted with the Director of Risk Management (DRM), who initially stated the incident with Client #1 was not a restraint, but on further questioning, the DRM confirmed the incident was a restraint. The DRM did not provide documentation of the restraint. On 08/15/2025 at 11:08 AM, during an interview Client #4 stated BHA #1 grabbed Client #1's arms and stated that Client #1 went "ragdoll" so BHA #1 drug Client #1 to his room. On 08/15/2025 at 12:10 PM, the Chief Operating Officer (COO) viewed the video with the surveyor and stated the interaction between Client #1 and BHA #1 would be an escort, but escorts are to be documented as a restraint. The COO confirmed there was not a physician order for the restraint, and the required documentation for the restraint was not completed. Client #2 During an interview on 08/14/2025 at 12:59 PM, Client #2 stated the class was getting a treat for good behavior, but Client #2 did not get one due to negative behaviors earlier in the day and would be required to watch the other clients eat their treat. Client #2 stated he wanted time away but couldn't get out of the classroom. A verbal exchange occurred with the Director Of Education , (DOE) then Client #2 pushed the DOE. The DOE then choked Client #2 and grabbed Client #2's legs after they both fell. Client #2 stated there were marks that showed the following day and was reported to BHA #3. On 08/14/2025 at 3:34 PM, during an interview	-501	501 Step #3: Systemic changes made to ensure deficient practice does not recur: - By 9/19/2025, all BHAs, Nurses, and teachers will be in-serviced on a patient escort being considered a restraint and ensuring all required documentation for that restraint is completed. - On 9/17/2025, the deficiencies noted will be a topic of discussion in the all employee town halls. Step #4: Monitoring Observations will be made through monthly camera review and restraint packet audits to ensure all restraint documentation is completed. Date of completion: September 19th, 2025	

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-501	Continued From page 6 the DOE stated Client #2 pushed her, so she put her hand on Client #2's chest and pushed the client into a chair. She then tripped and grabbed the client's leg. The DOE denied an attempt to bite the client. The DOE stated Client #2 did not get a treat and had to watch the other clients eat their treat to teach the client what to do and what not to do. The DOE stated that she "agitated and she was aggressive" during the incident. On 08/14/2025 at 2:25 PM, during an interview BHA #3 stated the DOE made Client #2 sit and watch the other clients eat a treat while the DOE verbally engaged with the other clients. Client #2 became more upset. BHA #3 stated the DOE turned around and "went after" Client #2, which caused them to fall into a filing cabinet. Client #2 and the DOE fell to the ground with the DOE's hands around Client #2's neck. BHA #3 tried to pull Client #2 off the ground but stated the DOE held Client #2's leg and could feel resistance while trying to lift him up. BHA #3 stated that after staff were able to separate them the DOE verbally engaged with Client #2 which resulted in Client #2 requiring a restraint. BHA #3 stated that throughout that classroom period the DOE antagonized Client #2, and it was fueling the situation. BHA #3 confirmed there were red marks present on Client #2 neck area the following day. On 08/15/2025 at 9:35 AM, during an interview BHA#4 stated the DOE wanted the clients who were not getting treats to watch the other clients eat theirs. Client #2 asked for a "Time Away", but the DOE denied the request and Client #2 bumped into the DOE. BHA #4 stated the DOE reacted by grabbing Client #2's neck and they fell to the ground at which point BHA #3 tried to pick Client #2 up, but the DOE was attempting to grab	-501		

Arkansas State Agency

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-501	Continued From page 7 Client #2's leg and pulled him back down. BHA #4 stated Client #2 went after the DOE again which required a restraint, and the DOE told the class, "this is what happens if you buck up against them". BHA #4 stated the DOE caused the situation by being aggressive, not letting Client #2 take a "Time Away" and not letting the BHAs do their job so the restraint could have been avoided. On 08/15/2025 at 10:38 AM, during an interview BHA #5 stated Client #2 and the DOE were antagonizing each other verbally and Client #2 threatened to hit the DOE. The DOE told Client #2 she wanted Client #2 to do it. The DOE then grabbed Client #2 by the neck and choked the client. They both fell to the floor while the DOE was still holding the client. BHA #3 tried to pull Client #2 up but the DOE tried to pull the client down. On 08/15/2025 at 10:57 AM, during an interview Client #5 who was in the classroom at the time of the incident, stated that Client #2 was trying to fight the DOE when the DOE said she wanted Client #2 to come over to her. The DOE grabbed Client #2 by the neck and started walking with Client #2 when they both fell on top of each other. Staff attempted to move Client #2 away from the DOE who was holding Client #2's leg. Client #5 stated that it was crazy and traumatizing to see. A review of a Witness Statement, dated 8/12/25 from the DOE, indicated Client #2 pushed the DOE and while moving Client #2 back to the front of the room toward staff, they slipped and BHA #4 and BHA #5 placed Client #2 in a restraint. A review of a Witness Statement from BHA #3, dated 08/12/2025, noted Client #2 bumped into the DOE, and the DOE grabbed Client #2 by the	-501		

Arkansas State Agency

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-501	Continued From page 8 neck, they both fell to the ground and while trying to lift Client #2 from the ground the DOE continued grabbing Client #2's leg. A review of a Witness Statement from the BHA #4, dated 8/12/25 noted Client #2 was aggressive with the DOE and verbally threatening when the DOE grabbed the client by the neck, which caused them to fall to the floor and while attempting to separate them the DOE grabbed Client #2's ankles. A review of a Witness Statement from BHA #5, dated 8/12/25, noted Client #2 bumped against the DOE and was being verbally aggressive when the DOE grabbed him around the neck. BHA #5 stated staff intervened and while separating the two when the DOE grabbed Client #2 by the ankles. A review of the Nurse's Notes dated 8/12/25 for Client #2 which noted there were no marks on Client #2's neck or throat and there was no complaint of pain. There was no entry on 08/13/2025 for evaluation of the red marks reported to BHA #3. A review of Client #2's file on 08/14/2025 did not reveal a physician order for a restraint and failed to reveal the required restraint documentation for the physical restraint conducted on 8/12/25.	-501			
-505 SS=D	Post-intervention debriefings (a)(1) Within twenty-four (24) hours after the use of restraint, staff involved in an emergency safety intervention and the Resident must have a face-to-face discussion. (2) This discussion must include all staff	-505			

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-505	Continued From page 9 involved in the intervention except when the presence of a particular staff person may jeopardize the well-being of the Resident. (3) Other staff and the Resident's parent or legal guardian may participate in the discussion when it is deemed appropriate by the Psychiatric Residential Treatment Facility. (4) The Psychiatric Residential Treatment Facility must conduct such discussion in a language that is understood by the Resident's parent or legal guardian. (5) The discussion must provide both the Resident and staff the opportunity to discuss the circumstances resulting in the use of restraint and strategies to be used by the staff, the Resident, or others that could prevent the future use of restraint or seclusion. (b) Within twenty-four (24) hours after the use of restraint, all staff involved in the emergency safety intervention, and appropriate supervisory and administrative staff, must conduct a debriefing session that includes, at a minimum, a review and discussion of: (1) The emergency safety situation that required the intervention, including a discussion of the precipitating factors that led up to the intervention; (2) Alternative techniques that might have prevented the use of the restraint; (3) The procedures, if any, that staff are to implement to prevent any recurrence of the use of restraint; and (4) The outcome of the intervention, including any injuries that may have resulted from the use of restraint. (c) The staff must document in the Resident's record that both debriefing sessions took place and must include in that documentation the names of staff who were present for the	-505		

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-505	Continued From page 10 debriefing, the names of staff that were excused from the debriefing, and any changes to the Resident's treatment plan that result from the debriefings. Authority. Arkansas Code §§ 20-76-201 et seq., and 25-10-129. This ELEMENT is not met as evidenced by: Based on observation of video, interview and record review, the facility failed to complete post-intervention debriefings as required after a restraint, for 2 (Client #1 and Client #2) of three clients reviewed. A review of Client #1's file on 08/14/2025 did not reveal documentation of a client post intervention debriefing or a staff debriefing for a physical restraint observed on video to have been conducted by Behavioral Health Associate (BHA) #1 on 08/01/2025. On 08/14/2025 at 2:42 PM, an interview was conducted with the Director of Risk Management (DRM) who initially stated the incident with Client #1 was not a restraint, but on further questioning, the DRM confirmed the incident was a restraint. The DRM did not provide documentation of the restraint. On 08/15/2025 at 12:10 PM, the Chief Operating Officer (COO) viewed the video with the surveyor and stated the interaction between Client #1 and BHA #1 would be an escort, but escorts are to be documented as a restraint. The COO confirmed there was no documentation of the post intervention debriefing for the client or debriefing for the staff after the restraint occurred. A review of Client #2's file on 08/14/2025 did not	-505	505 Step #1: Corrective Action - On 8/8/2025, the employee involved in the incident with Client #1 was placed on administrative leave and was terminated on 9/4/2025. - On 8/14/2025, the employee involved in the incident with Client #2 was placed on administrative leave. - On 9/17/2025, the deficiencies noted will be a topic of discussion in the all employee town halls. - By 9/19/2025, all BHAs, Nurses, and teachers will be in-serviced on a patient escort being considered a restraint and ensuring all required documentation for that restraint is completed. Step #2: Identification of others with a potential to be affected by deficient practice: - On 8/8/2025, through census review, there were 164 other clients within the PRTF with the potential to be affected by the deficient practice noted. Of the 164, 16 clients were in the unit of the BHA involved in the incident with Client #1. - On 8/12/2025, through census review, there were 170 other clients within the PRTF with the potential to be affected by the deficient practice noted. Of the 170, 14 clients were in the classroom where the incident with Client #2 took place.		

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-505	Continued From page 11 reveal documentation of a post-intervention debriefing for the client or staff after the restraint on 08/12/2025. On 08/14/2025 at 2:25 PM, during an interview BHA #3 stated there was only documentation of the restraint required following the altercation between Client #1 and the DOE, not of the hands around his neck and them falling.	-505	505 Step #3: Systemic changes made to ensure deficient practice does not recur:		
-601 SS=D	Resident Rights (a)(1) The Psychiatric Residential Treatment Facility shall have written policies and procedures defining the rights and responsibilities of Residents that comply with 42 C.F.R. § 483.356 (2001). (2) The policies shall present a clear statement defining how Residents are to be treated by the Psychiatric Residential Treatment Facility, its personnel, and others involved in providing care. (3) All policies shall be available for review upon request by a Resident or a Resident's parent or legal guardian. (b) A copy of the synopsis of the Residents' bill of rights must be prominently displayed within the Psychiatric Residential Treatment Facility. This shall include: (1) Restraint and seclusion policy for the protection of Residents. (2) Each Resident has the right to be free from restraint or seclusion, of any form, used as a means of coercion, discipline, convenience, or retaliation. (3) An order for restraint must not be written as a standing order or on an as-needed basis. (4) Restraint must not result in harm or injury of the Resident and must be used only:	-601	- By 9/19/2025, all BHAs, Nurses, and teachers will be in-serviced on a patient escort being considered a restraint and ensuring all required documentation for that restraint is completed. - On 9/17/2025, the deficiencies noted will be a topic of discussion in the all employee town halls. Step #4: Monitoring Observations will be made through monthly camera review and restraint packet audits to ensure all restraint documentation is completed. Date of completion: September 19th, 2025		

Arkansas State Agency

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NAME OF PROVIDER OR SUPPLIER HABILITATION CENTER, LLC		STREET ADDRESS, CITY, STATE, ZIP CODE 1810 INDUSTRIAL DRIVE FORDYCE, AR 71742		
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-601	Continued From page 12 (A) To ensure the safety of the Resident or others during an emergency safety situation; and (B) Until the emergency safety situation has ceased and the Resident's safety and the safety of others can be ensured, even if the restraint or seclusion order has not expired. (c)(1) Upon admission to the Psychiatric Residential Treatment Facility, staff must provide the Resident and his or her parent or legal guardian with a statement of rights and responsibilities and of all rules and regulations governing Resident conduct and responsibilities. (2) The Psychiatric Residential Treatment Facility is to communicate these expectations and rights during the period of, not more than two (2) weeks before, or five (5) working days after, admission, unless medically contraindicated in writing. (3) The Psychiatric Residential Treatment Facility shall obtain a signed acknowledgement from the Resident and his or her parent or legal guardian. (4) The acknowledgement is maintained in the Resident's medical record. Authority. Arkansas Code §§ 20-76-201 et seq., and 25-10-129. This ELEMENT is not met as evidenced by: Based on video review, interview and record review the facility failed to protect 2 of 3 clients reviewed from verbal and physical abuse from peers and staff. On 8/14/25 at 2:42 PM a review of video surveillance was conducted for a restraint which occurred on 08/01/2025 in the unit's dayroom. The video footage revealed between 2:58:20 PM	-601	601 Step #1: Corrective Action - On 8/8/2025, the employee involved in the incident with Client #1 was placed on administrative leave and was terminated on 9/4/2025. - On 8/14/2025, the employee involved in the incident with Client #2 was placed on administrative leave. - On 9/17/2025, the deficiencies noted will be a topic of discussion in the all employee town halls. - By 9/19/2025, all BHAs, Nurses, and teachers will be in-serviced on Zero Tolerance for abuse and neglect. Step #2: Identification of others with a potential to be affected by deficient practice: - On 8/8/2025, through census review, there were 164 other clients within the PRTF with the potential to be affected by the deficient practice noted. Of the 164, 16 clients were in the unit of the BHA involved in the incident with Client #1. - On 8/12/2025, through census review, there were 170 other clients within the PRTF with the potential to be affected by the deficient practice noted. Of the 170, 14 clients were in the classroom where the incident with Client #2 took place.	

Arkansas State Agency

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-601	Continued From page 13 and 3:01:23 PM Client #1 was repeatedly moving toward another client in an aggressive manner. Client #1 grabbed a broom and shook it at the other client. At 3:01:28 PM Client #1 was standing still and did not have the broom in the Client's hands. Behavioral Health Associate (BHA) #1, walked into the view of the camera. At 3:02:37 PM BHA #1 was observed grabbing Client #1 with both hands on the client's shoulders, turned Client #1 around and pulled both of Client #1's arms behind the Clients' back. BHA #1 walked from the dayroom to the client's room, holding Client #1's arms behind the clients back. Between 3:02:37 PM and 3:309 PM BHA #1 lifted Client #1 off the ground. On 08/14/2025 at 12:44 pm, during an interview Client #1 stated BHA #1 let several kids beat him up because BHA #1 thinks Client #1 is rude to him and doesn't listen. Client #1 stated that BHA #1 said if Client #1 did not tell anyone BHA #1 would buy him fast food. On 08/15/2025 at 11:08 AM, during an interview Client #4 stated that BHA #1 grabbed Client #1's arms identified it as a hold and stated that Client #1 went ragdoll so BHA #1 had to drag Client #1 the rest of the way to his room. Client #4 stated that BHA #1 called them and several other clients to the room to be witnesses. They decided that Client #1 was being disrespectful to BHA #1 so they all went in at which time Client #1 slapped Client #4 and he and Client #5 and Client #6 started punching Client #1. Client #4 stated that when they started punching Client #1, BHA #1 tried to separate them immediately. On 08/14/2025 at 3:15 PM, during an interview BHA #1 stated that Client #1 was having behaviors with another client, so BHA #1 escorted	-601	601 Step #3: Systemic changes made to ensure deficient practice does not recur: - By 9/19/2025, all BHAs, Nurses, and teachers will be in-serviced on Zero Tolerance for abuse and neglect. - On 9/17/2025, the deficiencies noted will be a topic of discussion in the all employee town halls. Step #4: Monitoring Observations will be made through monthly camera review and restraint packet audits to ensure all restraint documentation is completed. Date of completion: September 19th, 2025	

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-601	Continued From page 14 him to his room. BHA #1 stated a restraint packet was not completed at the time and did not know if one was completed. BHA #1 stated an escort was not considered be a restraint. On 08/14/2025 at 12:59 PM, during an interview Client #2 stated that the class was getting a treat for good behavior, but they would not get one due to negative behaviors earlier in the day and the DOE told them that they would have to watch the other clients eat their treat. Client #2 stated he wanted time away but couldn't get out of the classroom and after a verbal exchange with the Director Of Education (DOE) Client #2 pushed the DOE and the DOE choked them and grabbed their legs. Client #2 stated that they did not have any marks when the RN checked him but that there were marks the following day that they showed to BHA #3. On 08/14/2025 at 2:25 PM, during an interview BHA #3 stated the DOE made Client #2 sit and watch the other clients eat a treat while the DOE verbally engaged with the other clients. Client #2 became more upset. BHA #3 stated the DOE turned around and "went after" Client #2, which caused them to fall into a filing cabinet. Client #2 and the DOE fell to the ground with the DOE's hands around Client #2's neck. BHA #3 tried to pull Client #2 off the ground but stated the DOE held Client #2's leg and could feel resistance while trying to lift him up. BHA #3 stated that after staff were able to separate them the DOE verbally engaged with Client #2 which resulted in Client #2 requiring a restraint. BHA #3 stated that throughout that classroom period the DOE antagonized Client #2, and it was fueling the situation. BHA #3 confirmed there were red marks present on Client #2 neck area the following day.	-601			

Arkansas State Agency

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-601	Continued From page 15 On 08/15/2025 at 9:35 AM, during an interview BHA#4 stated the DOE wanted the clients who were not getting treats to watch the other clients eat theirs. Client #2 asked for a "Time Away", but the DOE denied the request and Client #2 bumped into the DOE. BHA #4 stated the DOE reacted by grabbing Client #2's neck and they fell to the ground at which point BHA #3 tried to pick Client #2 up, but the DOE was attempting to grab Client #2's leg and pulled him back down. BHA #4 stated that Client #2 went after the DOE again which required a restraint, and the DOE told the class, "this is what happens if you buck up against them". BHA #4 stated the DOE caused the situation by being aggressive, not letting Client #2 take a "Time Away" and not letting the BHAs do their job so the restraint could have been avoided. On 08/15/2025 at 10:38 AM, during an interview BHA #5 stated Client #2 and the DOE were antagonizing each other verbally and Client #2 threatened to hit the DOE. The DOE told Client #2 she wanted Client #2 to do it. The DOE then grabbed Client #2 by the neck and choked the client. They both fell to the floor while the DOE was still holding the client. BHA #3 tried to pull Client #2 up but the DOE tried to pull the client down. On 08/15/2025 at 10:57 AM, during an interview Client #5 who was in the classroom at the time of the incident, stated that Client #2 was trying to fight the DOE when the DOE said they wanted Client #2 to come over to them and the DOE grabbed Client #2 by the neck and started walking, they both fell on top of each other and when staff was trying to move Client #2 away the DOE was reaching for Client #2's leg. Client #5	-601		

Arkansas State Agency

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-601	Continued From page 16 stated that it was crazy and traumatizing to see. During an interview on 08/14/2025 at 1:48 PM, the Chief Operating Officer stated there was no video of the incident that occurred in the classroom on 08/12/2025 as that camera was not working that day. The DOE stated they consistently have problems with their surveillance system and are discussing implementation of a new system.	-601		